Surviving Sepsis Campaign International Guidelines

for the Management of Septic Shock and Sepsis-Associated Organ Dysfunction in Children (2020)

Weiss SL, Peters MJ, Alhazzani W, et al. Society of Critical Care Medicine (SCCM) and European Society of Intensive Care Medicine (ESICM). Published in Pediatric Critical Care Medicine and Intensive Care Medicine, February 2020.

This document contains selected extracts of the 2020 Pediatric Surviving Sepsis Campaign guidelines relevant to emergency department and pre-hospital pediatric triage.

Section reserved for full guideline text

The complete 2020 Pediatric Surviving Sepsis Campaign guidelines span approximately 50 pages and cover diagnosis, resuscitation, hemodynamic support, ventilation, corticosteroids, blood products, nutrition, and endocrinology. This extract focuses only on the sections relevant to the RORY pediatric triage decision support system.

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§ 3.4 Age-Adjusted Vital Sign Thresholds for Sepsis Screening

School-age pediatric patients (6-12 years) with sustained heart rate above 120 bpm in the absence of fever source warrant sepsis screening.

Age-band reference ranges (PALS 2020):

Infants (0-12 months): HR 100-160, RR 30-60
Toddlers (1-3 years): HR 90-150, RR 24-40
Preschool (3-6 years): HR 80-140, RR 22-34
School-age (6-12 years): HR 70-120, RR 18-30
Adolescents (12-18 years): HR 60-100, RR 12-20

Tachycardia exceeding the upper bound by more than 10% in the absence of a clearly benign cause (anxiety, exercise, pain) shall be considered a positive screening criterion.

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§ 5.2 Fever and Documented Wound Source

The combination of fever and documented wound source in a pediatric patient is a high-suspicion pattern for invasive bacterial infection and requires mandatory workup.

Any pediatric patient presenting with fever (temperature greater than 100.4 F / 38 C) and a history of skin breakdown, scrape, laceration, or other potential portal of entry within the preceding 7 days shall be evaluated for sepsis using the full screening protocol. This combination - fever plus portal of entry - was present in the 2012 Staunton case and remains one of the highest-sensitivity early indicators of pediatric sepsis.

Documentation shall include: location and age of the wound; signs of local inflammation; presence or absence of purulent discharge; and any patient-reported worsening since onset.

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§ 6.1 Altered Mental Status Criteria

Altered mental status in a febrile pediatric patient - including language such as 'confused,' 'won't make eye contact,' or 'not acting right' - shall be considered a qSOFA-equivalent criterion.

Nursing and triage staff shall document verbatim any parental or caregiver descriptions of behavioral change. Phrases including but not limited to: 'not himself,' 'looks weird,' 'unusually quiet,' 'won't respond,' 'mottled,' 'pale,' and 'different from normal' shall be flagged as potential altered mental status indicators and shall trigger sepsis screening regardless of objective Glasgow Coma Scale score.

These subjective descriptors are frequently the earliest available indicators of sepsis in pre-verbal or school-age children and often precede measurable changes in vital signs by 4 to 12 hours.

§ 7 Fluid Resuscitation Timing

In children with septic shock, administration of 40-60 mL/kg of isotonic crystalloid within the first hour is recommended in settings with availability of intensive care.

§ 8 Antibiotic Timing

Empiric broad-spectrum antibiotics shall be administered as soon as possible after recognition of sepsis, ideally within 1 hour of identification.